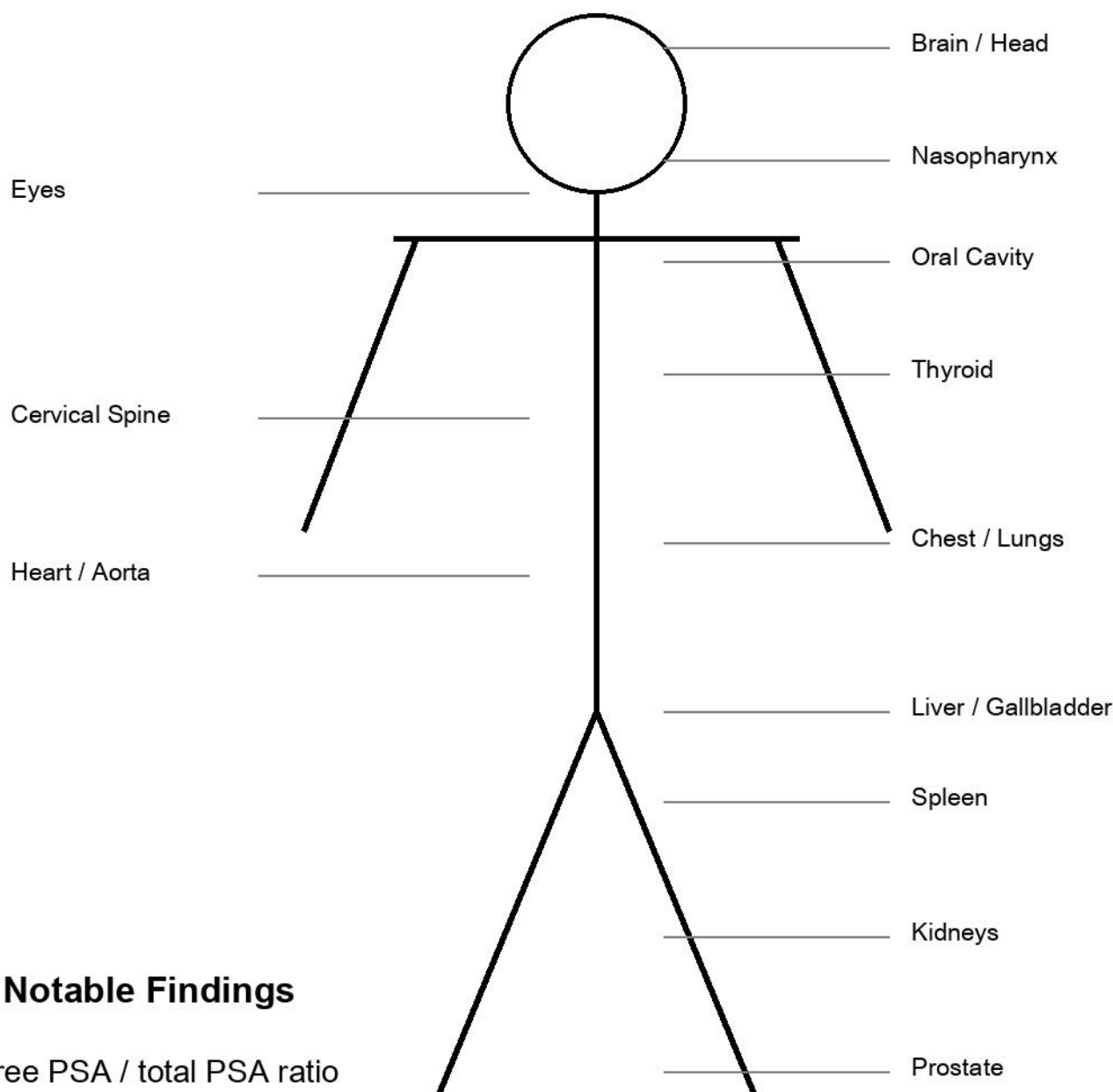


Medical Examination Report

Professional English Translation - Clean Version

Patient: Su Tieshan
Exam Date: 12 June 2025
Issuing Institution: iKang Junan (Shanghai iKang Junan Outpatient Clinic Co., Ltd.)
Document Type: Health Check / Medical Examination Report
Note: This is a clean English-only translated edition. All Chinese text and marks have been removed.



Overview of Notable Findings

1. Decreased free PSA / total PSA ratio
2. Obesity with increased waist circumference
3. History of hypertension; blood pressure normal at this examination
4. Sinus bradycardia
5. Prominent aortic arch
6. Elevated indirect bilirubin
7. Decreased plateletcrit
8. Mild fatty liver
9. Bilateral renal crystals
10. Cervical degenerative hyperostosis / osteophyte formation
11. Status post anal fistula surgery
12. History of appendectomy
13. Refractive error
14. Increased cup-to-disc ratio in both eyes

Interpretive Notes

Dear Mr. Su Tieshan,

Thank you for visiting iKang Junan (Shanghai iKang Junan Outpatient Clinic Co., Ltd.). Enclosed is your medical examination report for 12 June 2025.

This report reflects the findings of the specific items examined during this visit and does not represent a complete assessment of every organ system or all possible health indicators. Conclusions are based on the health information provided by the examinee together with the findings from this examination. Different examination methods may yield different results for the same organ or system.

Major Abnormal Findings, Follow-up and Treatment Advice

1. Decreased free PSA / total PSA ratio
2. Obesity; increased waist circumference
3. History of hypertension; blood pressure normal at this examination
4. Sinus bradycardia
5. Prominent aortic arch
6. Elevated indirect bilirubin
7. Decreased plateletcrit
8. Mild fatty liver
9. Bilateral renal crystals
10. Cervical osteophyte formation / cervical degenerative hyperostosis
11. Status post anal fistula surgery
12. History of appendectomy
13. Refractive error
14. Increased cup-to-disc ratio in both eyes

Specialist Recommendations and Guidance (Part 1)

1. Decreased f-PSA / t-PSA ratio

Prostate-specific antigen is a protein secreted mainly by prostatic epithelial cells. When total PSA is 4-10 ng/mL, the free-to-total PSA ratio may assist in differentiating prostate cancer from benign prostatic hyperplasia. If both total PSA and free PSA are elevated and the f-PSA/t-PSA ratio is reduced, further urologic evaluation may be considered if clinically indicated.

2. Obesity; increased waist circumference

A body mass index of 28 or above is consistent with obesity. Increased waist circumference suggests central adiposity. Lifestyle optimization is recommended, including calorie control, a lower-fat diet, adequate protein intake, increased vegetables and fruit, smoking cessation, alcohol limitation, and regular exercise.

3. History of hypertension; blood pressure normal at this examination

Hypertension is a chronic multifactorial disease. Regular blood pressure monitoring and follow-up with a cardiologist are recommended. Continue medication as prescribed and do not discontinue therapy without medical advice. A low-salt, low-sugar, low-fat diet is advised.

Specialist Recommendations and Guidance (Part 2)

4. Sinus bradycardia

Sinus bradycardia refers to a heart rate below 60 beats per minute. In many cases it is of limited clinical significance, especially in individuals who exercise regularly; however, further cardiology review is appropriate if symptoms such as palpitations, chest tightness, dizziness, or dyspnea occur.

5. Prominent aortic arch

This may be associated with longstanding hypertension or constitutional factors. Management should focus on the underlying cause, with cardiology review if symptoms occur.

6. Elevated indirect bilirubin

Mild elevation is often of limited clinical significance, but repeat testing is recommended. Persistent elevation may warrant specialist assessment.

7. Decreased plateletcrit

Clinical correlation is recommended; repeat testing may be performed if necessary.

8. Mild fatty liver

This indicates mild fat accumulation in hepatocytes. Weight management, physical activity, and periodic follow-up are recommended.

Specialist Recommendations and Guidance (Part 3)

9. Bilateral renal crystals

Adequate hydration is recommended. Small asymptomatic crystals may not require intervention, but larger crystals or symptoms such as renal colic should prompt urologic review.

10. Cervical degenerative hyperostosis / osteophyte formation

This is generally related to chronic strain, aging, and degenerative change. Management depends on severity. If headache, dizziness, or upper-limb numbness occurs, specialist review is advised.

11. Status post anal fistula surgery

Specialist follow-up is recommended if symptoms recur.

12. History of appendectomy

No specific intervention is required unless symptoms develop.

13. Refractive error

Routine ophthalmologic follow-up is advised.

14. Increased cup-to-disc ratio in both eyes

Further ophthalmologic evaluation, including intraocular pressure and visual-field assessment, is recommended to exclude glaucoma.

Medical Examination Results - General Examination and Internal Medicine

Height: 179.0 cm

Weight: 91.5 kg

Body Mass Index (BMI): 28.6

Systolic Blood Pressure: 129 mmHg

Diastolic Blood Pressure: 84 mmHg

Waist Circumference: 100 cm

Interpretation: Obesity; increased waist circumference

History: 5-year history of hypertension, on treatment

Pulse: 66 beats/min

Heart rhythm: Regular

Cardiac examination: Normal

Lung auscultation: No abnormal breath sounds heard bilaterally

Liver: Not palpable below the costal margin

Renal angle tenderness: None

Other internal medicine findings: None

Physical Examination and Ophthalmology

Skin: No obvious abnormality detected

Head and neck: No obvious abnormality detected

Extremities / musculoskeletal screening: No obvious abnormality detected

Anorectal / genital examination: Declined

Uncorrected visual acuity: Right 0.8, Left 0.8

Color vision: Normal

Fundus: Enlarged physiologic optic cup in both eyes; cup-to-disc ratio > 0.5

Non-contact intraocular pressure: Right 14 mmHg, Left 15 mmHg

Fundus photography: Retinal vessels and macula unremarkable; increased cup-to-disc ratio in both eyes

Impression: Increased cup-to-disc ratio in both eyes; refractive error

ENT, Complete Blood Count and Urinalysis

ENT examination: No obvious abnormality detected in the external ears, auditory canals, tympanic membranes, nasal septum, pharynx, or tonsils. Hearing by tuning fork was normal.

Complete Blood Count:

WBC $7.84 \times 10^9/L$

RBC $5.10 \times 10^{12}/L$

Hemoglobin 158 g/L

Hematocrit 0.47 L/L

MCV 91.8 fL

MCH 31.0 pg

MCHC 338 g/L

RDW-CV 12.8%

Platelets $152 \times 10^9/L$

MPV 10.7 fL

PDW 12.6 fL

Lymphocytes 34.80%

Neutrophils 56.6%

Absolute lymphocytes $2.73 \times 10^9/L$

Absolute neutrophils $4.44 \times 10^9/L$

RDW-SD 41.9 fL

Plateletcrit 0.16% (low)

P-LCR 30.6%

Monocytes 5.6% / $0.44 \times 10^9/L$

Eosinophils 2.2% / $0.17 \times 10^9/L$

Basophils 0.8% / $0.06 \times 10^9/L$

Urinalysis:

Specific gravity 1.030

pH 6.0

Leukocytes, nitrite, protein, glucose, ketone, urobilinogen, bilirubin, and occult blood: Negative

Urine RBC 0 /HP, urine WBC 0 /HP, casts 0 /LP

Appearance: Clear; Color: Light yellow

Biochemistry, Tumor Markers and Additional Laboratory Tests

Urine microalbumin: 7.2 mg/L
AFP 2.41 ng/mL
CEA 1.78 ng/mL
ALT 21 U/L
AST 24 U/L
GGT 15 U/L
ALP 67 U/L
Total bilirubin 20.17 umol/L
Direct bilirubin 5.82 umol/L
Indirect bilirubin 14.35 umol/L (mildly elevated)
Total protein 74.1 g/L
Albumin 44.7 g/L
Globulin 29.40 g/L
A/G ratio 1.5
Total bile acid 5 umol/L
Cholinesterase 8201 U/L
Urea 5.12 mmol/L
Creatinine 92.00 umol/L
Uric acid 332.00 umol/L
Fasting blood glucose 4.36 mmol/L
Total cholesterol 5.01 mmol/L
Triglycerides 1.03 mmol/L
HDL-C 1.55 mmol/L
LDL-C 3.44 mmol/L
hs-CRP 0.60 mg/L
Calcium 2.31 mmol/L
Phosphorus 1.05 mmol/L
Magnesium 1.00 mmol/L
Total PSA 1.560 ng/mL
Free PSA 0.327 ng/mL
f-PSA/t-PSA ratio 0.21 (decreased)
CA19-9 12.92 U/mL
NSE 9.51 ng/mL
CYFRA21-1 2.00 ng/mL
FT3 1.62
Sodium 138.40 mmol/L
TSH 2.48 mIU/L

ECG, Ultrasound, Radiology and Final Imaging Summary

Electrocardiogram: Sinus bradycardia; heart rate 57 beats/min.

Abdominal ultrasound: Sonographic findings consistent with mild fatty liver. Spleen morphology and echotexture were normal. Punctate hyperechoic foci were seen in both kidneys, consistent with bilateral renal crystals. Intrarenal blood flow was normal.

Prostate ultrasound: Prostate normal in shape and size; capsule clear; internal echogenicity homogeneous; no obvious abnormality detected.

Thyroid ultrasound: Both lobes normal in size and shape; capsule intact; internal echogenicity homogeneous; blood flow normal on Doppler.

Carotid ultrasound: Bilateral carotid luminal diameter normal; intima-media not thickened; intima smooth; blood flow normal.

Echocardiography: Aortic root 28 mm; left atrial diameter 35 mm; interventricular septum 10 mm; left ventricular posterior wall 10 mm; LVEDD 46 mm; LVESD 27 mm; main pulmonary artery 19 mm; left ventricular ejection fraction 71%; fractional shortening 40%. Cardiac chamber sizes were normal, wall motion was normal, and no obvious valvular or Doppler abnormality was detected.

Chest radiograph: No focal pulmonary opacity. Lung markings clear. Cardiac silhouette not enlarged. Prominent aortic arch noted.

Cervical spine radiograph: Physiologic curvature preserved. Marginal osteophyte formation at C3, C5, C6 and C7 with increased osseous density. Calcification of the anterior longitudinal ligament noted.

Impression:

1. Prominent aortic arch
2. Cervical osteophyte formation / cervical degenerative hyperostosis
3. Bone density: Normal
4. Stool routine and stool occult blood: Not performed because examination was declined.